

CREATING AWARENESS

The health sector is on a roll with schemes in place to widen the reach of medical services

A professional trying to become a politician. That is the best description of the 39-year-old Union Minister for Health and Family Welfare, Dr Anbumani Ramadoss. An alumnus of Madras Medical College, this Puducherry-born medico is the youngest minister in the United Progressive Alliance (UPA) cabinet. Well-acquainted with the lack of good healthcare facilities at the grassroot level in villages and districts across the country, he is working on bringing in reforms in the sector. In an interview, he talks about the National Rural Health Mission (NRHM). Excerpts:

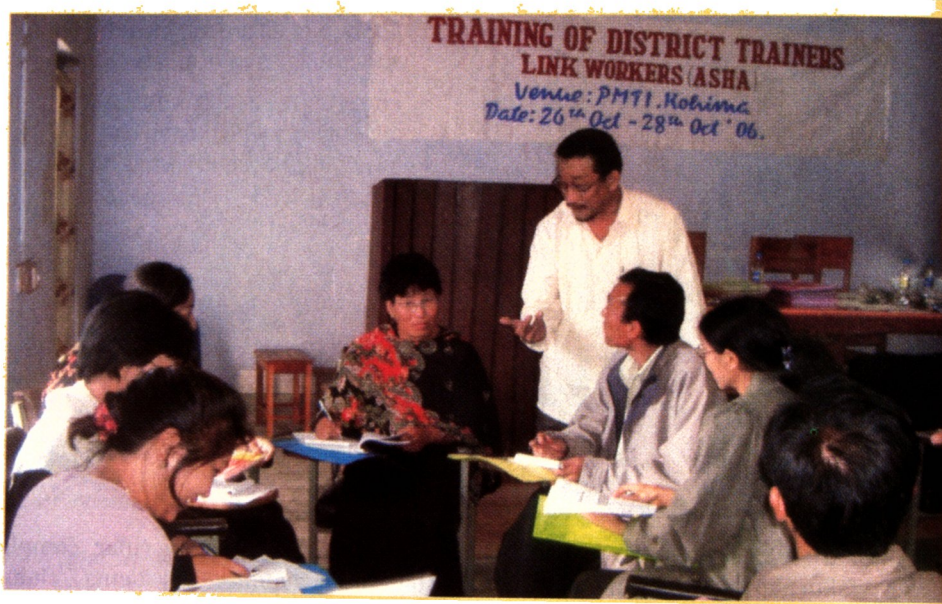
Q: Why is there a need for NRHM?

A. After taking over as Health Minister, during my first meeting with the Prime Minister, it was advised that the mission of the government and health ministry will be priority to rural health. It took about eight months to conceptualise the NRHM programme before it was launched by the Prime Minister on April 12, 2005. It is not just another project; it is a seven-year mission programme and is different from other projects we had till now. As you know, about 73 per cent of our people live in rural areas but they have access to only 25 per cent of medical facilities.

So, NRHM is directed towards providing accessible, affordable and accountable quality health services to the poorest households in the remotest regions of the country. As the reach of health services is not adequate, the mission's focus is on establishing a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence at all levels.

Q: How is NRHM different from other health schemes?

A. In a sector known for its fragmentation and compartmentalised perspectives, NRHM has provided a platform for community health action at all levels. Through a concerted effort at decentralised planning and through preparation of District Health Action Plans, the programme has managed to bring about intra health sector and inter-sectoral con-



NRHM—5 MAIN APPROACHES

COMMUNITISE

- Set up hospital management committee/Panchayati Raj Institutions (PRIs) at all levels.
- Give united grants to community/PRI bodies.
- Allot funds, functions and functionaries to community bodies.
- Decentralise planning, village health and sanitation committees.

FLEXIBLE FINANCING

- Partnership of state and community resources.
- United grants to institutions.
- NGO sector for public health goals
- Risk pooling—money follows patient.
- More resources for more reforms.

IMPROVED MANAGEMENT THROUGH CAPACITY

- Block and district health office with management skills.
- NGOs in capacity building.
- NHSRC/SHSRC/DRG/BRG.
- Continuous skill development support.

INNOVATION IN HUMAN RESOURCE MANAGEMENT

- Nurse managers.
- More nurses—local resident criteria
- 24X7 emergencies by nurses at primary health centres, AYUSH.
- 24X7 medical emergency at community health centres.
- Multi-skilling.